

HP: Imprinting the Global Health Sector

In 2009 Hewlett-Packard (HP) was undertaking a strategic transformation in the way it approached and implemented corporate social responsibility (CSR). The company had established a new team, called the Office of Global Social Innovation (OGSI), which was tasked by HP's board to reimagine the company's CSR work. The sixty-nine-year-old company had weathered numerous technological and strategic changes over the years, and had emerged as an industry leader in computer hardware, software, and IT services. It also had a long-standing reputation for philanthropy and CSR, but these areas had recently moved to the back burner as the company endured a turnaround and multiple leadership changes.

Gabi Zedlmayer, a twenty-two-year HP veteran, was the vice president of the new OGSI team, and one of the key principals on the team was Paul Ellingstad, a twelve-year HP veteran. Zedlmayer and the OGSI team wished to reimagine HP's social and environmental contribution in a way that was strategically aligned with the goals of the changing business. They wanted to go beyond CSR, to reach further than foundation giving programs and employee volunteer days. They wanted to implement projects that would have true strategic importance for HP. And they sought to design a strategy that would be customer-focused, enabling the company to move into new markets and engage with new partners to co-create innovative products and services that would add value to the business.

HP's board was in agreement with the principles of this new approach but wanted to see it in action. It asked the OGSI team to develop a pilot project to demonstrate the company's shift from traditional CSR to social innovation. Zedlmayer tasked Ellingstad with designing this initial project. He had six months to explore the challenge of developing a project that would bring HP's strategic competencies and growth strategies to bear on an important social challenge. On January 30, 2010, he would have to present his suggested course of action to HP's board of directors.

The Hewlett-Packard Company

Bill Hewlett and Dave Packard met at Stanford University before forming a two-person company in a rented garage—the original Silicon Valley startup. But this was not the 1990s, or even the 1980s; this was in 1939. The Hewlett-Packard Company grew its product offering and global presence over the years through acquisitions and joint ventures.

Long known as an industry leader in hardware products, the company was rooted in its founders' values of trust, respect, achievement, integrity, teamwork, flexibility, and innovation.

Between 2005 and 2008, however, HP was facing financial and reputational challenges. Carly Fiorina, who had been CEO since 1999, was forced to resign in January 2005 as a result of inconsistent profits and problems with executing the acquisition of Compaq. The highly controversial move to acquire Compaq in 2001 led to a nearly year-long proxy battle with the families of Compaq's founders. Additionally, HP's inability to match personal computer (PC) manufacturer rival Dell's growth and profitability also contributed to Fiorina's dismissal.¹

Fiorina was succeeded by chief financial officer Robert Wayman, who served as interim CEO in 2005, from February 10 to March 28, when Mark Hurd was brought from the outside to lead the company. Hurd took on the challenge of turning around the company, as HP was struggling to compete with IBM's high-end services business and Dell's low-cost PC operations.²

Unfortunately for Hurd, reputational concerns stemming from the drawn-out Compaq acquisition and leadership changes continued to be an ongoing issue in addition to financial challenges. On September 5, 2006, *Newsweek* revealed that HP's general counsel, at the request of HP board chairwoman Patricia Dunn, had contracted a team of private investigators to follow board members and several journalists.³ The board members were outraged that their privacy had been compromised, and many resigned on the spot. Despite these challenges, Hurd weathered the storm and successfully led the organization through a strategic transformation, addressing operational and financial challenges. Hurd's strategy was primarily focused on cost cutting across every phase of global operations. After years of missing Wall Street expectations, under Hurd HP met expectations in twenty-one out of twenty-two quarters and increased profits for twenty-two straight quarters, while its revenue rose 63 percent and stock price doubled. Key operational measures, such as inventory turns, also indicated that the company was improving.

Industry Overview: Hardware, Software, and IT Services

As of 2009, HP was a multinational technology company offering a range of hardware (e.g., computers, printers, and data centers), software (e.g., human resources and payroll software and enterprise applications), and services (e.g., business process outsourcing, networking, and technology consulting) in the information technology industry. Customers included individual consumers, small- and medium-sized businesses, and large enterprises, including customers in the government, health, and education sectors.⁴ HP had operations in the Americas, Europe, Middle East, Africa, and Asia-Pacific.

In 2008 and 2009, HP was the clear industry leader in terms of global PC sales, with 20.4 percent worldwide market share as of 2009 (see **Figure 1**). This was an improvement over its 2008 market share of 18.9 percent. HP had a 7 percent share advantage over Dell as of 2009, which marked improvement from a 4 percent advantage for HP in 2008. HP sold about two-thirds of its PCs outside the United States, and the overseas PC market growth was stronger. Dell was ahead of HP in terms of U.S. sales, and the two remained strong rivals.⁵

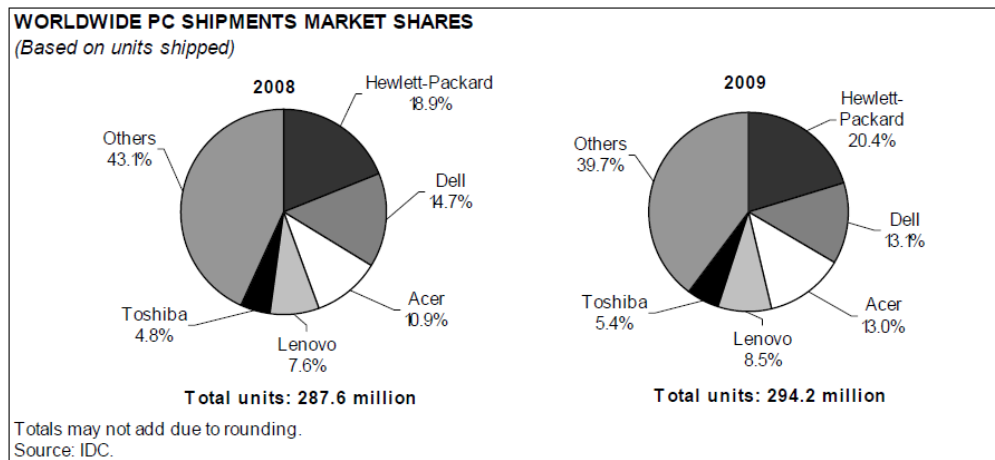
¹ Scott Morrison, "HP Results Show Little Improvement in Net Profit," *Financial Times*, February 17, 2005.

² Scott Morrison, "HP Set to Name NCR Chief New CEO," *Financial Times*, March 29, 2005.

³ Alan Murray, "Directors Cut: H-P Board Clash Over Leaks Triggers Angry Resignation," *Wall Street Journal*, September 6, 2006.

⁴ Hewlett-Packard, Marketline Report, May 23, 2014.

⁵ Standard & Poor's Industry Survey: Computers: Commercial Hardware, October 2009.

Figure 1: PC Market Share, 2008 and 2009

Source: Standard & Poor's Industry Survey: Computers: Hardware, April 2010.

There was widespread industry transformation and consolidation happening across the IT market. HP sought to expand its portfolio from hardware to a bigger footprint and capability in technology services, a common trend across the industry. According to Standard & Poor's, "as computer hardware has increasingly become a commodity, manufacturers have looked for other ways to leverage their base of intellectual expertise. In recent years, many have begun to focus on opportunities for growth in services markets, particularly as demand for traditional hardware has slowed. Their heightened focus on services has intensified competition in the market."⁶

During Hurd's tenure, HP made numerous acquisitions, including the purchase of IT services provider Electronic Data Services (EDS) on August 26, 2008, for \$13.9 billion. The acquisition was, by value, the largest in the IT services industry and the second largest in the technology sector, only behind HP's prior acquisition of Compaq.⁷ Following the integration of EDS, HP's profile would resemble IBM's, enabling the company to become more of a one-stop-shop provider to its clients, offering hardware, services, and software. Before the merger, IBM was the largest independent IT services company, while EDS and Accenture, which were about the same size, were tied for second place.⁸

Developing a New Strategy for Social and Environmental Impact

HP had long been committed to corporate social responsibility. Employees often harked back to the early words of their founders, such as David Packard's 1947 statement, "The betterment of

⁶ Standard & Poor's Industry Survey: Computers: Commercial Services, November 2008.

⁷ "HP Completes \$13.9 Billion Acquisition of EDS," press release, August 26, 2008, <http://www.hp.com/hpinfo/newsroom/press/2008/080826xa.html>

⁸ Standard & Poor's Industry Survey: Computers: Commercial Services, November 2008.

society cannot be left to a few; it is a responsibility to be shared by all. HP exists to create a value for shareholders but also exists to create a value for society.”⁹

In recent years, including under Fiorina’s tenure, HP was transforming and innovating within the philanthropy space—with its work on digital inclusion, for example. However, with the leadership transitions and Hurd’s focus on operational and financial challenges, the company had not dedicated as many resources to its philanthropic work. Hurd did not see CSR or philanthropy as essential to the business, and said the company did not have time “for hobbies.”¹⁰ However, this did not go unnoticed by HP’s board.

The board was of course happy with the turnaround that Hurd had led, but it felt that the company’s CSR work had been ignored in the process. At the time CSR, which was part of the marketing team, was seen as a cost center. The board, feeling that CSR should be more central to HP’s work, asked Hurd to do something about it. At the time, the marketing team, led by chief marketing officer Michael Mendenhall, was undergoing a wider transformation from a three-layer organization (global, regional, and country-level) into a centralized, smaller marketing organization. In July 2009, Hurd asked Mendenhall to specifically rethink HP’s corporate philanthropy approach and strategy, which was then undertaken by the social investment team. As part of this transformation, the incumbent head of social investment left the company and Mendenhall asked Gabi Zedlmayer, who had been running EMEA (Europe, Middle East, and Africa) corporate marketing, to run the new social investment team. Zedlmayer asked Ellingstad to join and help them reimagine what CSR should mean for HP.

Prior to joining the team, Ellingstad worked for HP’s head of enterprise sales in the EMEA region, running customer programs and communications. Through various engagements with foundations outside of work, he also had a personal interest and experience in philanthropic work.

The new OGSi team sought to develop a social responsibility strategy that was more strategically aligned with HP’s business. The team had seen how CSR had gone by the wayside when the company had to rein in spending. CSR and philanthropic activities in the company were viewed as costs rather than as strategic levers, and therefore were seen as competing with revenue-generating and cost-cutting activities. The team knew, however, that there was potential for this type of work to bring greater value to HP.

To help it develop a new strategy for CSR, the team sought an outside perspective from UK-based consulting firm Volans, which focused specifically on strategies for social and environmental impact. Volans co-founder and director Charmian Love said, “HP found us. Paul [Ellingstad] called us one day and said he had seen a report we had published called *50 Pioneers in the Business of Social Innovation*. It was about the type of economy that we felt, from interviews and work with world leaders, would rise from the ashes after the financial meltdown of 2008–2009. It talked about a shift to social innovation, to a more long-term perspective and an economy with a more integrated, global, triple bottom line of economic, social, and environmental value added into its DNA.”¹¹

In the conversations with Volans, Ellingstad drew on the work he knew—HP’s go-to-market strategies. This included how new products and solutions were brought to market through working with “early adopter” clients and how close collaboration to establish a proof of concept

⁹ Georgiann Zaglakis, HP Communications, e-mail communication with the author, July 7, 2014.

¹⁰ Paul Ellingstad, in interview with the author, July 7, 2014.

¹¹ Charmian Love, in interview with the author, July 14, 2014.

or a showcase of the solution was a vital step in the market adoption and launch.¹² Showcase projects could expand and strengthen how HP was seen by potential clients.

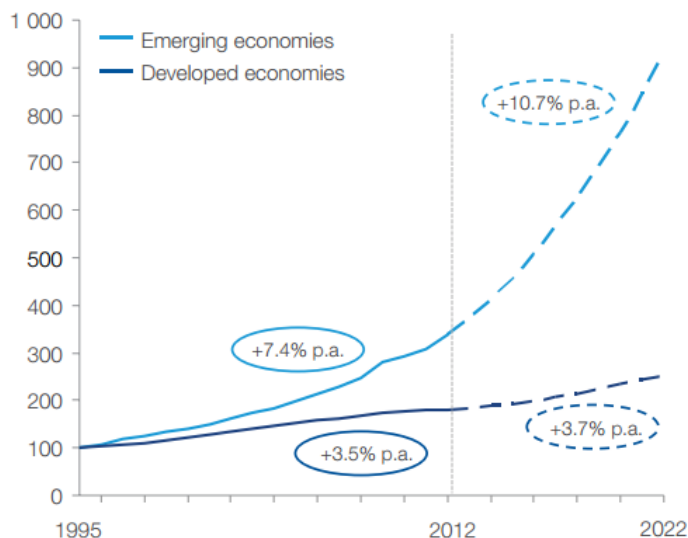
HP's prior philanthropy work had been almost solely focused on education. Instead of simply looking at expanding work within the education sector, the team engaged Volans in helping to think about a new strategy that aligned with HP's overall corporate strategy. Volans and the OGSi team therefore identified the business areas where HP was strong and also where the company was planning to grow. What they realized was that HP was actually the biggest hardware and software provider in the healthcare sector. The EDS acquisition had contributed to this strength, as EDS had been the biggest business process outsourcer for Medicare and Medicaid billing within the United States. So from a commercial point of view, the healthcare sector was already an incredibly important area for the company in terms of current position and future growth. Globally, it was calculated to be a \$65 billion total addressable market, compared to the \$29 billion education market.¹³

Ellingstad's experience working in EMEA sales also contributed to his recognition of the importance of future growth in emerging economies. Although HP had offices in numerous emerging markets, the company was often just considered a printer manufacturer, with its enterprise services unknown to many potential customers in these markets. And when the team consulted statistics and industry data, the growth trend for expenditure on healthcare in emerging economies was promising (see **Figure 2**).

Figure 2: The World Economic Forum's Outlook on Global Health Expenditure

Sources: World Bank, Business Monitor International, BCG

Health expenditure (Index: 1995 = 100)



Note: The Y axis represents an index with 1995 expenditure as a baseline unit of 100.

Source: World Economic Forum, "Health Systems Leapfrogging in Emerging Economies: Project Paper," January 2014.

¹² Paul Ellingstad, in interview with the author, July 7, 2014.

¹³ Market share information courtesy of HP, internal confidential presentation, 2009.

At the same time, the team also studied the United Nations Millennium Development Goals (MDGs) (see **Figure 3**) to see where the greatest needs were in terms of human and environmental challenges. Of course, health goals ranked high on this list.

Figure 3: Millennium Development Goals



Source: United Nations, Millennium Development Goals and Beyond 2015, <http://www.un.org/millenniumgoals> (accessed August 31, 2014).

The team evaluated global social and environmental needs (using the MDGs as the primary, broadly accepted and recognized prioritization of these needs globally) and then set that beside HP's business priorities and objectives to discover where they overlapped. The team determined that healthcare was a key area in which HP's expanded CSR work could have a big impact. But the question of *how* the company should work was just as important as what area it should focus on moving forward. With the help of Volans, the team discussed its assets in terms of industry expertise, human capital, and technology. It did not just want to simply donate or invest money; it wanted to engage in the health solutions ecosystem and play an active role in solving problems and developing solutions that would have a direct and measurable effect in the global healthcare sector. The team designed an assessment framework to help identify the right projects to engage in moving forward, considering measures of business alignment, social impact, and the company's core competencies.

The CMO's Presentation to the Board

In September 2009, Michael Mendenhall, HP's CMO, presented the work his team had done to lay out a new strategy (called HP's Social Innovation) to the public policy committee of HP's board. The presentation included slides outlining OGSI's mission and charter, its rationale for targeting the healthcare sector, and key stakeholders for the sector (see **Exhibit 1**).

The presentation also included a benchmarking study that HP had undertaken that compared corporate giving across various organizations. It had measured giving in total as well as giving as

a percentage of profit, comparing HP's spend against that of its competitors and Fortune 500 companies, and comparing the spend of U.S.-based companies versus international companies. The study helped HP understand where it stood in relation to other organizations, as well as the range of expenditure across various companies.

The new strategy was designed to be better aligned with the company's overall growth objectives in healthcare and included an emphasis on building partnerships to enable HP to enter this new area with trusted allies. Partnership was a conscious choice and an important piece of the strategy.

The OGSi team would seek out projects that would be more akin to the go-to-market strategy that Ellingstad had experienced in enterprise sales work. And it would aim to engage in projects for which technology could transform the healthcare process, resulting in better outcomes for patients. In addition to presenting a strategy for engaging in healthcare in mature markets, a great deal of the team members' focus was on emerging economies. They talked about how "the future economies are growing out of developing markets today. That's where the real growth is coming from, and quite frankly, that's also where the innovation is coming from as well."¹⁴ As shown in **Figure 4**, the team would operate separately from the HP Foundation, with the foundation focused on disaster relief and coordinating employee giving and the OGSi team carrying out social innovation projects.

Figure 4: Social Innovation Organizational Structure



Source: HP Annual Global Citizenship Report 2010.

¹⁴ James Militzer, blog entry, "‘There Is No Element of the Market That Is Really Safe’: Hewlett Packard’s Paul Ellingstad on Health Care Innovation, and Doing Business at the BoP,” *NextBillion*, August 20, 2013, <http://www.nextbillion.net/blogpost.aspx?blogid=3450>.

The board reacted favorably to the theory behind the new strategy and gave Mendenhall its approval to proceed. It felt that realigning this work with HP's business goals made sense. The OGSi team would continue to report to the CMO but would have a limited budget, and it would have to build alliances both within HP and with organizations outside the company in order to successfully execute the new strategy.

Another consideration was the large number of well-intentioned projects, especially those in technology, that were later held up as examples of failure in the developing world. Laptops had gone unused; equipment had been installed with no one receiving proper training on its use; and centers had been built in places that did not have the electricity to support their sophisticated equipment. How would HP avoid the mistakes that others had made in "parachuting in" a technology in the developing world? How could it understand this new set of customers' needs when its customers in this engagement might consist of rural, distributed individuals living in poverty without easy access to education or technology?

The board directed Mendenhall to develop a concrete proposal for projects that would kick off the work of this new team under its new strategy. The board's public policy committee, a committee of board members that focused on policy issues, had oversight and reviewed progress as the work of the new OGSi team unfolded under the CMO.

Developing the Pilot Project

Zedlmayer, Ellingstad, and the OGSi team wasted no time. With the new CSR strategy and structure approved, there was a vacancy to fill on the team, and Ellingstad knew who he wanted to fill it. Zedlmayer and Ellingstad called Aziz Mohamed, with whom Ellingstad had worked during his time on the sales team. They wanted to recruit Mohamed, who was working in corporate marketing, to come on as a project lead within the OGSi team, overseeing the first project that would be developed for the healthcare work. Mohamed was honored to be pulled into the global project, but he had never even heard the term "social innovation." He later said:

When I received a call from Gabi [Zedlmayer] and Paul [Ellingstad] and they were asking me to get involved in social innovation, to be honest I did not know what they were talking about. I had been in marketing for almost fifteen years. I knew about our philanthropic work, and I had helped out here and there, but I could not understand why they would call me about that. I didn't see a connection to the work I was doing. Of course, I then talked to them more and learned more about what they intended to do and it seemed like an exciting new project to get involved with. Once they explained the connection between HP's business and our potential for impacting society it made sense.¹⁵

Ellingstad and Mohamed started talking to country leaders in Africa, a region of the world that was an epicenter for global health projects. Thanks to their prior work in sales and marketing there, they understood the organizational structure and went first to the managing director of HP's East Africa office, Kenneth Mbwaya. They explained the new social innovation approach and Mbwaya helped them understand the region better, from the perspective of the business, the

¹⁵ Aziz Mohamed, in interview with the author, July 16, 2014.

capability on the ground in varying countries, and the opportunities for growth. Mbwaya welcomed the corporate initiative and saw it as a legitimate request from corporate leadership.

Ellingstad and Mohamed had to develop their framing for Mbwaya and subsequently for each potential partner and individual that they hoped to bring into the process for finding a project that could exemplify their new strategy. They needed to explain what they were looking for and what they wanted to accomplish. They outlined to Mbwaya, and later to potential project partners, that the priorities for this first project were a hybrid of meeting the greatest need with the greatest potential for success. In order to achieve this success, they felt that it made sense for the project to be in a relatively stable environment, with trusted partners on board, and relying on input and work from HP in an area in which it had expertise. Ellingstad and Mohamed wanted a project that would rely on HP's "bread-and-butter business," which they saw as a hardware solution.

They hoped the technology solution would help a process leapfrog. For emerging economies, the most valuable use of leapfrogging was not just to catch up with developed economies, but to use innovation that allowed them to take a shortcut in reaching a more advanced development stage without accumulating inefficiencies along the way.¹⁶

When talking to Mbwaya, Ellingstad and Mohamed thought it made sense to consider projects in Kenya both because of HP's existing presence in and knowledge of the country, and because of the social needs of the country.

Despite the economic and political stability in Kenya, there were a number of healthcare challenges in the country at the time. Generally, in the healthcare sector across Africa, qualified healthcare personnel were very scarce. The continent faced 28 percent of the global disease burden but had only 3 percent of the world's health workforce.¹⁷ In Kenya, a country with a population of 40 million, there were only around 1.5 health workers per 1,000 people, and many rural clinics did not have Internet access or even electricity.

Until that point, HP's business in Kenya primarily consisted of being a hardware supplier to the government and private industry, which purchased printers and laptops from the company. HP was not sought out as an advisor or partner in developing technology services, let alone as an expert in healthcare. Ellingstad knew that developing a project in this sector would raise a few eyebrows, as people might wonder what a printer company had to do with fighting HIV in Africa.

If HP wanted to impact the health sector in the country, it would have to change its image with the government and obtain a deeper understanding of the population it was seeking to help. For that, it would need a new partner to help legitimize its activity.

There was a complex web of organizations working on health in Kenya, as it was a fairly stable environment that unfortunately still had a great deal of dire health challenges. There were organizations with funding and research capabilities, such as USAID, the CDC, and the World Health Organization, as well as the Kenyan Ministry for Health. There were nonprofit initiatives such as Doctors Without Borders, which had hundreds of staff and had worked in Kenya since 1987, and World Vision, which had been working in the country since 1974. And there were multinational companies, such as GE Healthcare (GE Africa was headquartered in Kenya) and Phillips Healthcare, which was designing a portable mobile ultrasound system for rural health

¹⁶ World Economic Forum, "Health Systems Leapfrogging in Emerging Economies: Project Paper," January 2014.

¹⁷ Ibid.

clinics. The power of “m-health” (health services and diagnostics enabled by mobile technology) was also evolving, and local companies, such as mobile company Safaricom, played a role in this.

Ellingstad said, “I knew we couldn’t do this alone,” and he looked to Volans and other consultants for input and recommendations on partnerships and potential projects.¹⁸ Volans helped him identify organizations that were already working on global health in Kenya. What was the existing ecosystem, and what gap could HP fill? According to Charmian Love, Volans considered itself “a relationship broker . . . we have a wide network of contacts across various social and environmental issues, so we could help introduce HP to potential partners working on healthcare in Kenya.”¹⁹

Ellingstad knew of the Clinton Health Access Initiative (CHAI) organization, and Volans confirmed that it might make a good partner in the region. CHAI considered itself a government advisor on healthcare, with a focus on strengthening organizational systems. CHAI was born in 2002 out of the work of President Bill Clinton and the Clinton Foundation. The organization was part of Clinton’s initial post-presidency work in global health and was originally focused on reducing the cost of AIDS treatment drugs through negotiations with drug companies that included volume discounts. A key part of driving down the cost was ensuring that there would be a significant pool of patients on the demand side, which could really only be guaranteed through government systems and programs. Through this work, CHAI built close relationships with governments and international aid organizations working on health issues in Africa.

Approaching CHAI

Ellingstad approached CHAI’s head of corporate partnerships, expecting to receive a positive reception to his offer to help on a project in healthcare in Kenya. Although CHAI welcomed HP’s willingness to help, it assumed the relationship would work very much like its other corporate partnerships: HP would work in consultation with CHAI to pick a particular project to support, then it would provide funding for that project. In return, HP would receive public recognition and thanks for its financial support to the mission and work of CHAI.

CHAI Kenya deputy country director Jackson Hungu said, “[HP] came to us as a Fortune 500 company, and of course we were initially excited about the fact that they might offer us a pot of unrestricted funds. We rarely get that kind of money. So our initial approach was to wrangle over the size of the check. But HP insisted that they wanted to offer a different range of assets—from technology to product development and employee engagement. Once we recognized the value of this package, we agreed—but at first we did not understand their approach.”²⁰ In fact, it took Ellingstad three months to convince CHAI that HP wanted a different type of engagement.

He did not expect this to be a hold-up in his project development process, but he knew how important it would be to formalize this partnership. Once it was formalized, CHAI agreed to help HP identify a project that met its criteria—working in an area where there was a need for a technology solution, but doing so in a way that would be relatively straightforward and affordable and could be implemented quickly.

¹⁸ Paul Ellingstad, in interview with the author, July 7, 2014.

¹⁹ Charmian Love, in interview with the author, July 14, 2014.

²⁰ Jackson Hungu, in interview with the author, July 23, 2014.

CHAI Presents HP with Potential Projects

CHAI invited the OGSi team members to Boston to discuss various projects that HP could engage in. Mohamed recalled his thoughts as they sat down to listen to CHAI's presentation: "When we were starting out, we really asked ourselves how HP could contribute. What were we the best at? How could we do more than just give money away?"²¹ The team thought that by donating HP's skills and expertise, in addition to its money, it would have a greater impact. There would be a multiplying effect on the company's investment.

Among the projects that CHAI presented, one stood out clearly to the team: the issue of early infant diagnosis of HIV/AIDS in Kenya. CHAI had been part of a wider effort in the 2000s within the global health community to get infants diagnosed and treated as soon as possible, to reduce the number of deaths from the disease. Early diagnosis and commencement of treatment for infants who are HIV-positive is vital to their survival. The mortality rate for HIV-positive infants who go undiagnosed and untreated can be as high as 50 percent by the age of two.²² The type of test that needed to be used on infants was highly sophisticated and could realistically only be carried out in research labs.

Kenya's lack of healthcare providers and the rural nature of many of its clinics affected the process of analyzing HIV/AIDS test results for infants. In Kenya, where every year approximately 1.5 million infants were born, the transmission rate of HIV from mother to child could be between 35 and 40 percent, according to the Kenya Ministry of Health.²³

The existing process for testing infants for HIV in Kenya was to take blood samples at some 3,000 health clinics, then send them by mail to five national health laboratories to be tested, and then mail the test results back to hubs, where they were picked up periodically by the rural clinics.²⁴ The clinics were supposed to have the results for patients within thirty days of the testing, but because of the complex nature of the chain of logistics, often the information did not come within that timeframe—and sometimes it never came at all. Mothers would arrive at their thirty-day check-up, often traveling from afar, and be told that their test results were not yet available. They left frustrated and lost confidence in the clinic's ability to diagnose or treat their children. At best, their children would receive delayed treatment. At worst, the mothers would not return and their children would never receive treatment at all. This early infant diagnosis (EID) process was taking as long as several months to return results.²⁵

Jackson Hungu of CHAI said:

We worked very hard to build a system for infant testing that could link local health clinics, which were accessible to new mothers, with the sophisticated testing that needed to be carried out in the national laboratories. But we knew it was inefficient and that the incentives to have tests delivered on time were just not there. There were actually three challenges with the existing system. The first was that patients didn't get test results on time. The second was that the records in the labs themselves were very rudimentary—

²¹ Aziz Mohamed, in interview with the author, July 16, 2014.

²² Gabi Zedlmayer, "HP's Cross-Sector Health Partnership in Kenya," *Stanford Social Innovation Review*, April 25, 2012, http://www.ssireview.org/blog/entry/hps_cross_sector_health_partnership_in_kenya.

²³ Ibid.

²⁴ Eleanor Whitehead, "WEF Africa 2012: How Technology Changes Africa's Development Prospects," *This Is Africa*, May 1, 2012.

²⁵ Zedlmayer, "HP's Cross-Sector Health Partnership in Kenya."

*basically paper-based record books. So, if a clinic called to check on the status of a test it was very hard to see where it was in the testing process. And lastly, the government and aid organizations did not have any ability to aggregate the data to get a national picture of the disease. Which interventions to prevent mother-to-child transmission were working? Were there certain regions that had a higher or lower number of infants being tested? These were questions that everyone wanted answered, but there was no system tracking the information.*²⁶

The system was not working, but with a distributed network of clinics, many of which lacked dependable electricity or Internet connection, how could the information be delivered more reliably and timely?

Mohamed said, “I was there when CHAI presented us with various potential projects. When I heard about EID I was so excited, because it seemed like an area that could really benefit from technology and also save thousands of lives. I knew it would not be easy to bring together so many partners and people from across HP to change the existing process and technology, but it just seemed like something we could really do well.”²⁷

Ellingstad agreed, saying, “The EID project was the best overlap between what was needed and what we could offer . . . There were others that would have been longer-term engagements, and I thought we might progress to those in the future. But for the first project we needed one that could happen in a timely fashion to demonstrate our new strategy in social innovation.”²⁸

The team members left CHAI’s office in agreement that the EID project would make the most sense. They started to think about structuring the project. Should HP offer equipment? Services? Research and development support? Who else would they need to involve to ensure that the right ecosystem was built around the technology? How could they help without just parachuting in and dropping a technology that might not be appropriate or might not be adopted? The crucial factor would be the new system’s reliability. As long as the results came reliably, mothers would not lose faith in the system. And if the results could be tracked more easily, there would be more accountability and the government and partners could potentially assess the effectiveness of prevention efforts in different regions.

The OGSi team members also wanted to create something that would be replicable and scalable. They wanted Kenya to be the pilot, but they knew from CHAI that these same challenges with disease diagnosis existed across Africa. They hoped that the project would demonstrate to the Kenyan Ministry of Health that HP was a capable partner that it could rely on for future work, and show other leaders within Africa that HP had an expertise in the area of IT solutions for the healthcare sector.

Recommending the Project to Management

Ellingstad had to make a recommendation in the coming weeks. He had so many ideas, but would the company leaders get on board? The CSR strategy that HP had employed to date had

²⁶ Jackson Hungu, in interview with the author, July 23, 2014.

²⁷ Aziz Mohamed, in interview with the author, July 16, 2014.

²⁸ Paul Ellingstad, in interview with the author, July 7, 2014.

worked pretty well by all accounts. Management might think that it would just be easier to write a check to CHAI and let it carry out the work. HP was still focused on the financial turnaround and its numerous acquisitions. The EDS acquisition was expected to bring a lot more business in HP's core, more developed markets, and its primary hardware products were market leaders. Ellingstad anticipated that management might feel this project was an unnecessary distraction at a busy time.

Ellingstad stayed up late on a Wednesday night to prepare his presentation for management. He managed to outline his presentation by completing the headlines for each slide (see **Figure 5**), as he tried to anticipate the board's primary considerations and reservations. He knew this was half the battle.

Figure 5: Ellingstad's Slide Headers

- Why should we invest in social innovation, and how can we measure our work in this area?
- How did the OGSi team evaluate potential projects?
- The EID project:
 - The current problems with EID
 - Why EID?
 - The business case
 - The social impact
 - How we will address this issue
 - How we will evaluate our work

On Thursday, he called Mohammed to ask his opinion on the risks of engaging in the project and his perspective on the investment. He wanted to anticipate the questions of risk and whether or not this project would be the most effective use of corporate resources. What would be alternatives to achieve the same objectives of corporate reputation and market entry?

How should Ellingstad finalize his presentation and design an answer to the questions of project risks and returns?

Exhibit 1: Slides from OGSi Team Presentation, August 2009**FY10 Mission & Charter****Mission**

Increase HP's brand value and revenue through social investment programs that are aligned with our overall business strategy. The overarching objective is to contribute to the global community by enabling and accelerating the creation of social innovations.

Charter

- Run best-in-class social innovation initiatives
- Enable breakthrough social innovations in collaboration with leading change makers
- Champion collaboration models with internal and external stakeholders to maximise ROI
- Engage and inspire employees as ambassadors and volunteers
- Leverage and apply HP's leadership and expertise
- Demonstrate operational and financial excellence

Why Healthcare?

1. **Market opportunity and targeted industry for HP growth**
 - \$65B IT spend in H&LS in 2010
 - 5.1% CAGR 12/09 (highest industry growth rate)
 - HP technology used by 11 of the 13 Fortune 500 pharmaceutical and biotechnology companies and leading supplier of computing solutions for life sciences research
2. **Health is the key focus of social investment and allocation of funding**
 - Health is #1 allocation of marketplace giving (28% of total); followed by Education (27% {combined K-12 and Higher Ed})
 - Significant increase in focus on health through Health reform in US (including stimulus spending)
3. **Public-private partnership model in health parallels role and need for social investment alignment to business strategy**
 - Example: Case study: Rhode Island HIE (health information exchange) "currentcare" combining public and private stakeholders for 'anytime, anywhere healthcare program that delivers access to health information'.
4. **Opportunity in health to showcase HP innovation and leadership**
 - The HP Digital Hospital solution is a framework of hardware, software, partner point solutions and consulting services that lay the foundation for digital transformation.
 - The HP solutions for electronic health records help healthcare organizations use their data to improve health and streamline operations.
5. **The GSI play to include:**
 - Expanded test/reference market for new HP healthcare solutions: digital hospital, e-health records, etc.
 - Expand availability and implementation of health technology innovation to underserved / commercially unavailable markets/segments/customers
 - Connect greatest health innovation needs to the significant philanthropic and government funding available for health
 - "Prime the pump" to begin health technology transformation in marginal / second wave markets/segments/customers (support for foundation solutions, create market for incremental health technology services which are funded by customer or through partnerships or non-HP philanthropic sources)

Exhibit 1 (*continued*)**Key stakeholders for GSI in Health**

GSI role is to drive alignment and collaboration, bringing together the clients in need with the solutions providers and the organizations that fund solutions (above and beyond provision through conventional commercial business transaction)

